

Ebola Virus Disease

Update 16, June 2026

Ebola virus disease (EVD) is a potentially severe illness caused by the Ebola virus. Early stages of the disease are characterized by fever, chills, fatigue, headache, sore throat, malaise, muscle, joint and back pain. These symptoms can progress to vomiting, diarrhea, chest and abdominal pain with swelling in the stomach, severe weight loss, delirium, shock and in severe cases bleeding, organ dysfunction and death.

Ebola is contracted through direct contact of the mucous membranes or non-intact skin with blood or body fluids from an infected person or animal. Additionally, Ebola can be transmitted from contact with surfaces contaminated by body fluids such as clothes, bedding, medical equipment.

There are currently no reported Canadian cases related to any outbreaks, although Canadians do participate in responding to outbreaks as they occur around the world. The risk to most travelers is considered low, but it is important to be on the lookout for illnesses compatible with EVD and remain vigilant for other illnesses prevalent in the region (e.g., Malaria, Typhoid, etc.).

Consider Ebola when there is a **history of travel to an affected area** within the last 21 days or **known contact with a patient diagnosed with Ebola** (e.g., health care workers), lab worker handling Ebola virus or processing body fluids, time spent in a healthcare facility in an Ebola affected area where cases are being treated, contact with human remains of an Ebola suspected or known case, contact with bats, primates or wild animal bush meat from Ebola affected areas **and** the symptoms listed below:

What are the symptoms of Ebola?

Symptoms can begin 2 to 21 days after exposure

Initial symptoms include:

- Fever
- Chills
- Headache
- Sore throat
- Cough
- Weakness
- Myalgia
- Fatigue
- Muscle pain
- Joint pain
- Loss of appetite

Additional symptoms may include:

- Rash (Erythematous maculopapular rash on trunk)
- Nausea, vomiting and diarrhea
- Conjunctivitis (Red eye)
- Chest and abdominal pain
- Jaundice
- Swelling and pain in the stomach area
- Severe weight loss
- Delirium
- Shock
- Bruising
- Nosebleeds
- Hemorrhaging (inside or outside body)

The incubation period for EVD is 2 -21 days. Patients are not infectious until they develop symptoms, most often fever.

Person-to-person transmission occurs through direct contact of the mucous membranes or non-intact skin with infected blood/body fluids including urine, vomitus, and feces. Once symptoms appear, the risk of transmission increases throughout the illness, particularly when the patient is vomiting, having diarrhea, or hemorrhaging. People are not infectious during the incubation period unless symptoms develop.

What should I do if I suspect a person may be at risk for EVD?

All patients are screened by call takers either in Dispatch or the Patient Transfer Network to identify whether they are at risk of having Ebola (through travel to an affected country or contact with a known case of EVD).

When attending to a patient with a cough, vomiting or diarrhea, you should be following **routine practices and additional droplet precautions**:

- Wear fluid impermeable gown, gloves, fit tested N95 mask and full-face shield.
- Put a surgical mask on the patient.
- Provide timely care to patients in quarantine without symptoms using routine precautions. If you unexpectedly encounter a suspected EVD patient, stop and contact CliniCall immediately, perform a PCRA and escalate precautions as directed. Do not initiate enhanced PPE beyond your scope. IDCT teams are specially trained and equipped with advanced IPAC measures (e.g., extended-cuff double gloving, apron use) for high-risk patient transport.

As part of your point-of-care risk assessment, if a patient has any of the above symptoms **ask for any travel history** to affected countries or if they have had any contact with wild animals (such as contact with bats, primates or wild animal bush meat from Ebola affected area) or a symptomatic person suspected of having Ebola in the previous **21 days**.

If so:

- **Leave the immediate area** – have the patient stay in a comfortable position then wait outside and minimize contact with others in the home while waiting for technical and medical advice.
- **Contact BCAS dispatch immediately for advice**
- **Do not administer nebulized therapy**
- **Avoid direct contact with the patient or with the patient's blood or body fluids**, including medical equipment such as needles which may be contaminated.

BCEHS is continuing to work on ways of minimizing potential exposure and will provide updates as available.

More information is available at the [Provincial Health Officer](#) website and the [Public Health Agency of Canada website](#).

BCAS Dispatch Centre Numbers:

Kamloops: 844-371-5140 (toll free), 250-371-5140 (local)

Vancouver: 844-408-7692 (toll free), 604-708-7692 (local)

Victoria: 855-915-2252 (toll free), 250-915-2252 (local)

References:

[BC EVD Contact Investigation and Management Guideline Jan 30 2023.pdf](#)